

Emotion Regulation, Relationship and Therapeutic Change in Analytical Psychology and Contemporary Psychodynamic Approaches

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Abstract

All contemporary psychotherapies agree that (failing) emotion regulation is central to psychological disorders and that psychotherapy is about improving emotion regulation. In his research on the “emotion-laden” complex Jung put an emphasis on the role of failing emotion regulation in contributing to psychological disorders as well as to change in the process of psychotherapy, but he left this field of research and took a very different direction in favour of his archetype concept. Psychodynamic approaches generally argue that changes in emotion regulation are accomplished through corrective emotional experiences in the therapeutic relationship. Insights from affective neurosciences and attachment research have had a major influence on how the therapeutic relationship is constructed in contemporary psychodynamic approaches. There is a lack of similar developments in analytical psychology, which leads to substantial differences between the models of Jungian psychotherapy in contrast to other contemporary psychodynamic approaches. The implications of these differences for the practice of psychotherapy and especially the role of the therapeutic relationship are pointed out.

Keywords: affective neuroscience, analytical psychology, attachment research, emotion regulation, psychodynamic psychotherapy, therapeutic relationship

At the beginning of the 20th century, Jung earned his international reputation, even before he became linked to the psychoanalytic movement, with his research on the affective-laden complex. He could provide empirical evidence that there are unconscious factors, fuelled by strong emotions, which are not accessible to or controllable by consciousness but which have a major impact on cognitive functioning and behaviour (Roesler, 2017). On the other hand, Jung left this field of research—the association studies—quite early and put the emphasis on the concept of archetypes.

In the last years, the focus on emotion regulation has gained major importance in clinical psychology: the major schools of psychotherapy agree that emotion regulation is the decisive factor for psychotherapy. On the one hand, psychological disorders can be understood as specific impairments of emotion regulation; on the other hand, psychotherapy aims at an improvement of this emotion regulation. These views have changed the traditional orientation in cognitive approaches, which assumed that emotions are the results of cognitions, in the sense that the cognition is before the emotion and gives them their specific colouration. LeDoux (1998) could demonstrate that the relationship between cognitions and emotions is characterized by a complex reciprocity. The attempt to regulate problematic emotions by cognitive control has turned out to be limited, whereas the psychodynamic schools have always emphasized that the improvement of emotion regulation takes place within the therapeutic relationship, through an internalization of a successful interpersonal regulation of the client's emotions.

In this paper, the parallels as well as the differences in the view on emotion regulation in analytical psychology, in the other psychodynamic schools and in contemporary theories of emotion are investigated with special emphasis on how this impacts different approaches to psychotherapy and the role of the therapeutic relationship.

Complexes

Complexes are, according to Jung, affective laden unconscious factors. They are organized around strong emotions, and they trigger strong emotions when activated, which adversely influences behaviour, e.g., cognitive processing and memory (Jung, 1959, para. 4). Pathological complexes are what has to be focused on in psychotherapy, and they have formed through problematic relationship experiences with caregivers which results in unresolved emotions, or, to put it in contemporary terms, poor emotion regulation. The theory of complexes is well proven empirically, and it was already in Jung's time. Jung received international reputation as a scientist and researcher because of his studies with the Association experiment. Already in this research he was able to provide evidence for the existence of unconscious emotional factors which influence behaviour. In the last decades, more research was conducted which confirms the concept of complexes as unconscious emotional factors, for example in fMRI studies (for an overview see Roesler & van Uffelen, 2018). It is also confirmed by developmental and attachment research, which has demonstrated that attachment patterns are formed during development around emotional experiences with caregivers (see below). An example is A. Damasio's (2012) concept of somatic markers. The neuroscientist was able to demonstrate that there is an emotional memory system which stores emotional experiences. It does so by connecting them with somatic markers,

i.e., bodily feelings. If the person is again in a similar situation, emotional memory is activated via the somatic markers. This is useful as it can support decision-making if several options are available. If the earlier experience was connected with negative emotion, in a similar later situation this emotion is activated by somatic markers and serves as protection against decisions with painful consequences; the whole process operates on an unconscious level. This supports Jung's concept of complexes. It also explains why psychic disorders based on earlier painful experiences and the accompanying dysfunctional patterns in cognition and behaviour are so difficult to change by psychotherapy. To provide therapeutic change, psychotherapy has to reach the level of these embodied emotional memories.

Basic Emotions as Signalling Systems

The neuroscientist Joseph LeDoux (1998, 2012) and his research tradition argue that emotions are signalling or information systems which point to unmet needs in the organism. This is also the reason why there are innate specific emotions, as they point to basic needs essential for the survival of the organism (see below). Moser (2023) argues that the basic function of emotions in this information theory perspective is to cause an interruption in mental processing so as to regulate an internal or external relationship. In addition, there is also a totally unconscious emotion regulation on the limbic level, which guarantees a relatively automated behavioural response, in the sense of top-down control to down-regulate the amygdala. It seems that in strong pathological complexes these regulatory processes could not develop, or they break down, as a consequence of unresolved early experiences and the accompanying strong emotions (Panksepp et al., 2017).

Panksepp (1998, 2011) identified seven "basic affective systems" (the idea was already put forward by Darwin in 1871): SEEKING (expectancy), FEAR (anxiety), RAGE (anger), LUST (sexual excitement), CARE (nurturance), PANIC/GRIEF (sadness), PLAY (social joy). These basic emotion systems are in a certain sense lifesavers, since they circle around existential needs and situations. The theory is extremely well empirically founded. The mainstream in the neurosciences agrees with the basic tenets of this approach, which has been called neuroaffective theory or theory of basic emotional systems. Neuropsychological systems such as Panksepp's basic systems are called bottom-up processes in cognitive neurosciences; in humans they are regulated and modulated by top-down mechanisms, e.g., controlled by the prefrontal cortex, up to the point that they can be totally inhibited. These regulatory mechanisms are fully dependent on one's individual history of experiences, first of all in primary caregiver relationships.

Emotions are Relational

In line with Panksepp, Ekman (1992) identified basic emotion expressions in the face, which form universal specific signalling systems. He described practically the same basic emotions as pointed out above but put the focus in his research on the facial expressions of these emotions and how they can be identified by other humans. He found that these basic emotions are expressed by universal facial expressions which can also be found in primates. These emotions and the connected facial expressions have a specific physiology and a hardwired coherence between detecting a specific facial expression and the according emotional reaction. Since they are universal they are expressed and can be understood similarly in all cultures in the world. Now the interesting point is that even newborn babies are able to show these facial expressions and to read them correctly in the face of another person (Hardy 2009); they can even imitate them if seen on the face of another person (Meltzoff & Moore, 1977).

Based on these insights, Ekman developed a theory about the functions of these basic emotions and their expressions. Emotions inform the individual about their own current inner state and signal to others about unmet needs. Thus, emotions prepare the organism for action, they activate in a specific way. This activation is directed towards others, this is a biologically preformed general human attitude. Humans make use of others and the relationship to them to regulate their emotions and to communicate their needs, which, as far as it refers to the basic emotions, are directed to other persons (see Table 1). Thus, emotions are relationship oriented.

In this sense, emotions are a signalling system which plays a crucial role in self-regulation and relationship regulation.

This interpersonal system of emotion regulation was further investigated by the infant researchers in the context of the Boston Process of Change Study Group (Stern, 1985). They found that in infancy and early childhood the child and its caregiver together form a shared emotional regulation system which communicates via the universal basic facial expressions of emotions

Table 1. Core emotions are oriented towards emotionally relevant other, and lead to action tendencies.

Emotion	Need	Action tendency
Anger	Protection	Set limits, defend
Sadness	Contact	Seek consolation/comfort
Fear	(Emotional) Safety	Flee, seek support/safe place
Disgust, Shame	Separation	Exclude/Expel, hide, turn away
Surprise, Interest, Agitation	Stimulation	Explore, turn towards
Joy	Wellbeing	Open up, seek contact, share

(Tronick, 1989). The genetic makeup of humans is oriented towards interaction with other human beings and sociality—the “social embedded brain” (Northoff, 2015). The most elaborate theory resulting from these considerations is attachment theory and attachment research.

Attachment

Human infants would not be able to survive without the care of a more experienced adult who is able to regulate their basic physiological needs as well as their stress levels, because young infants cannot take care of these basic physiological and psychological needs by themselves. The early environment of evolutionary adaptiveness among humans required the basic ability to become emotionally attached in order to survive and enhance inclusive fitness. Since the environment of humans consists mainly of situations with a high need of cooperation and complex social interaction, these are the competencies which human children must be prepared for. Thus, the evolutionary adaptiveness of humans involves the acquisition of complex social and interactive competencies, which is only possible through experiences in a social network itself, it cannot be pre-formed by genes. The genetic makeup only prepares the infant to be highly sensitive for social interaction (Simpson & Belsky, 2018). Following this model, it also can be assumed that the mating model of long-term pair bonds seems to be the most reasonable, since it provides the most suitable environment for the development of complex social competencies. On the other hand, this implies that the quality of the environment early in life can exert long-lasting effects on psychosocial development, including social competency and the development of specific mating and parenting strategies in both males and females.

On the other hand, depending on the quality of these early social interactions provided mainly by the caregivers, ...

Infants differ rather drastically in the quality of their attachment relationships, and attachment theory hypothesizes that this attachment performance is largely, albeit not exclusively, environmentally determined ... as a consequence of childrearing experiences with parents and other caregivers.... The parallel to language development is useful here. Every child is born with the capacity to learn a language, but the specific language environment determines the kind of language to be learned. (Bakermans-Kranenburg & van IJzendoorn, 2018, p. 155)

The findings of attachment research point to the fact that the development of the self of human beings is almost exclusively based on interpersonal processes. The newborn infant has just some basic abilities to draw the caregiver into a form of interaction, e.g., by imitating the face expression (therefore Ekman’s universal face expressions), or giving other signals which are then interpreted by the caregiver as an initiative to communication. Here,

the basic emotion systems play the role of a signalling system by which the infant informs the caregiver about its current needs. But this system relies almost totally on a caregiver who is aware of the signals and responds to them in a sensitive way. If this happens, the child can increasingly rely on the presence of the caregiver and use the adult person to regulate its needs and emotions. The way in which the adult caregiver responds, i.e., sensitive or not, will determine the internalized representations of these interactions (Daniel Stern: representations of interactions generalized). So, the self which is formed in such processes is constituted by interpersonal processes, by interactions. There is no way that the child could develop his/herself individually and independently. Even when children have formed an early form of the self, they still apply what is called social referencing: when confronted with a new experience, the first thing they do is to look on the face of the caregiver for emotional cues as to how to interpret this experience. These ideas have been put together in the theory of the social brain (Pfaff, 2013), which basically points out that the brain of humans is largely a social construction.

Consequences for Contemporary Psychoanalytic Theories

From these insights, in psychoanalysis, the relational and intersubjective schools have developed, which are based on the infant research conducted by the Boston Process of Change Study Group, employing concepts such as Stern's representations of interactions that have been generalized (Stern, 1985). The general idea here is that the psyche is structured and practically built from the early interactional experiences of the child in its relationships with caregivers. It also has to be noted that the term drive, so fundamental to orthodox psychoanalysis, meanwhile has been discarded in the contemporary schools of the Freudian tradition, and has been reformulated as the concept of motivations (e.g., Lichtenberg, Lachmann & Fosshage, 2009):

Post-Freudians, for instance, have replaced Freud's pleasure principle as the primary biological drive with the idea of the need for relationship and relatedness as our most basic human longing. Wherever we turn across the psychodynamic spectrum, we see therapy more concerned with engagement, and more critical of all forms of disengagement, especially narcissism and other disorders of relatedness. (Tacey, 1998, p. 228)

In this view, emotions are linked to motivations and thus serve to satisfy basic needs. These basic motivational systems include: Physiological regulation, Sensuality/Sexuality; Aversion; Caregiving; Attachment; Affiliation; Exploration (Lichtenberg, Lachmann & Fosshage, 2009); alternatively: Attachment/Safety, Exploration, Autonomy, Sexuality, Caregiving (Bischof-Köhler, 2011); the last

author adds some specifically human motivations: reflective functioning, religion/meaning, self worth.

Emotions are signalling systems to inform the organism in direction to specific motivations; they activate the organism in a specific way and at the same time inform the social environment about the current inner state of the person. This enables humans to show affective attunement and, on a higher level, mentalization. In this sense, the basic emotions are primarily adaptive, which means, when they are activated they lead to adaptive behaviours, individually as well as socially. Therefore, contemporary psychotherapy approaches, as for example Emotionally Focused Therapy, focus on these basic or primary emotions as their activation is in itself therapeutic.

For relational analysts, the social aspect of our being is fundamental to psychological life—it is not an add-on or a separate domain of our existence. This leads to a critique of what Stolorow and Atwood call “the myth of the isolated mind”.... It is not simply that the mind develops in the social context of relationships with others; there is a more fundamental idea here that the mind itself is social and that the private subjective self, the intrapsychic inner world, is subsequent to and contingent upon the relational context in which it is embedded. (Colman, 2018, p. 130)

Archetypes and Emotions: Jung’s Developmental Model

These ideas are more or less the contrary of how Jung imagined the development of a person, putting the emphasis mainly on what comes out of the individual (“the myth of the isolated mind”). Archetypes here are considered to be preformatted patterns and structures, which develop independently from experiences in interactional relationships (Roesler, 2021), what Jung calls “autochthonous development”. In contrast to the above model, Jung conceptualizes psychic energy, libido, as a general, undifferentiated emotional energy, which is behind all psychological processes. The archetypes provide the specific colouration to this general undifferentiated emotional energy.¹ Jung’s libido concept has parallels to the physiological tradition of emotion theories (Benecke, 2014), of which the most prominent is the so-called two-factor-theory of emotion by Schachter and Singer (1962). It argues that emotion is a combination of an unspecific physiological arousal which gains its specific colouration through a cognitive interpretation determined by the situational context. In contrast to this, the state-of-the-art in contemporary emotion theories is that basic emotions are specific from the beginning. Thus, contemporary psychoanalytic viewpoints have discarded all concepts of libido, they provide no additional insight and have no relevance for clinical work—concepts of energy have to be replaced

¹ “... if the image is charged with numinosity, that is, with psychic energy, then it becomes dynamic and will produce consequences.” (Jung, 1976, para. 589).

by concepts of affect/emotion, which are in line with contemporary research (Benecke, 2014).

For Jung, archetypes are channels for emotions or psychic energy in general, which give the energy a certain shape and direction (Jung, 1976, para. 589). In Jung, the general idea is that archetypes shape a universal, preformatted process of transformation in which emotions are the energy. Jung believed that the archetypes connected with emotions are similar to instincts in animals, in the sense that a specific symbol (representing an archetype) triggers a specific meaning or emotion, in the same manner as the innate release mechanism in behavioural biology triggers an instinctually fixated pattern of behaviour. Contemporary neuroscience contradicts this notion: there are no universal signals triggering specific emotions in humans. Emotions in humans are always the result of subjective interpretations, based on earlier experiences and individual emotion regulation. In *Ethology/Behavioural Biology* (Bischof, 1985/2020) an instinct/pattern of behaviour is clearly defined and has three components:

1. a trigger mechanism (innate release mechanism IRM), which means that there is a specific stimulus in the environment which can be detected by the individual, which then
2. activates a drive, which leads to
3. an inherited automatic coordinated action pattern.

There is no such thing in human beings (for more details see Roesler, 2023).

It also has to be noted that the findings around these basic emotion systems clearly show that, in contrast to Jung's thoughts (and as argued recently by Alcaro et al., 2017), the genetic information does not directly activate behaviour patterns, but it activates affect and emotion, which is consciously experienced and then initiates actions.

In this context, it is again important to consider that much of the massive neocortical expansions since human divergence from greater apes was substantially controlled by a single gene variant, ARHGAP 11 B. We think such findings provide little leeway for robust and specific genetic control of various high-end neocortical primary sensory and affective functions [...] (Panksepp et al., 2017, p. 199)

So, there is no direct path from genes to behaviour, but the path goes from genes via emotions to behaviour (Solms, 2015). The fixated patterns also do not react to trigger stimuli, as instinctual behaviour in animals does. In contrast to instincts, the basic emotion systems in humans are signalling systems for existential needs and lead to typical behaviour directed mostly towards the caregivers, e.g., attachment behaviours. The basic emotion systems are neither connected with images or narratives, nor do they form connections with higher order processes, such as cultural expressions, as has been argued by Jung regarding archetypes:

Nonetheless, [the instinct] is psychologically important because it leads to the formation of structures or patterns which may be regarded as determinants of the human behaviour.... The immediate determining factor is not the ... instinct but the structure resulting from the interaction of instinct and the psychic situation of the moment. The determining factor would thus be a modified instinct.... Instinct ... would play the role of a stimulus merely, while instinct as a psychic phenomenon would be an assimilation of this stimulus to a pre-existing pattern. A name is needed for this process. I should term it psychicization. (Jung, 1969, para. 234)

Contemporary theories directly contradict Jung's conception of basic patterns which then evolve into individual representations, images, narratives etc. in the course of life—as is suggested in the above quote—instead, all higher cognitive processes are totally dependent on experience and learning, which mainly takes place in relationships to primary caregivers (for an extended discussion see Roesler, 2023).

Consequences for the Practice of Psychotherapy

The above insights, of course, have far-reaching implications for the practice of Jungian psychotherapy. In contrast to the classical school, where the therapist waits for an autochthonous development to unfold out of the person independently, based on biologically/genetically imprinted archetypes, should the emphasis not be much more on creating a certain quality of relationship, as the relational schools in psychoanalysis suggest? But if so, what is the relationship between the idea of archetypes and an archetypal development of the person in the course of psychotherapy on the one side, and the contribution of the therapeutic relationship on the other? It seems to me that this relationship is not really clarified even in contemporary schools of Jungian psychology.

It is interesting that recent accounts of the process which is assumed to unfold in Jungian psychotherapy often largely ignore the classic model of archetypes. For example, Kast (1999), in her ambitious attempt to reformulate the process in Jungian psychotherapy, successfully integrated the insights of infant observation and the concept of “representations of interactions that are generalized” (Boston Change Process Study Group, Stern 1985), but describes the psychotherapeutic process, although she calls it archetypal, without any reference to the classic archetypes. The same applies to the publications by Mario Jacoby (1998), who also attempted to integrate the Jungian approach with the findings of the infant researchers and Kohutian self-psychology; the term archetype is almost totally abandoned, the conceptualization of the “self” follows Stern (1985), not Jung. The only reference to the term archetype is to the “creative and ordering factor, which is called archetype” (Jacoby, 1998, p. 89; author's translation).

The Role of the Therapeutic Relationship

In Jung's perspective, development of the personality happens almost exclusively from inside the individual, autonomously, relationships do not play a role beyond their being a projection screen (Healy, 2017). For example: "At the heart of marriage is the question whether one can live his true nature, and if one can give the other—being it husband or wife—the freedom for their individuation" (Jung in Jaffé, 2021, p. 57; author's translation). This means a relationship, at best, is not an obstacle to individuation.

This attitude is in drastic contrast to the state-of-the-art in human and social sciences (the "relational self"), as presented above—and in practically all the other psychodynamic schools—that relationships are at the very beginning of individual development and are absolutely essential for the development of the personality. There is a lack of clarity in the theory of analytic psychology as to where development comes from, which role relationships play in the matter and what that means for the therapeutic relationship.

Analytical Psychology as elaborated by Jung and his immediate followers did not focus on the depth psychological aspects of the early infant and childhood development. Neither was there much attention paid to the usefulness of understanding the varieties of relationship that can occur in the consulting room between patient and analyst.... The lack of a clinical and theoretical tradition of investigation in these two important areas ... with the resulting lack of interest in understanding their interrelationship via the analysis of the infantile transference, left AP impoverished in an important way. This would need to be rectified if AP was to go on developing as a creditable professional and clinical endeavor. (Solomon, 1997, p. 119)

There has been some theoretical development regarding the conceptualization of the transference and its role in psychotherapy, mainly in the British—so-called developmental—school of analytical psychology—but there is no match to the theoretical developments in the Freudian tradition (e.g., object relations theory, self-psychology, infant observation etc.). In this model of the British school (Solomon, 1997), in contrast to Jung, the role of interaction with caregivers and their capacity to help and support the infant in reintegration becomes crucial; this role can also be taken over by the analyst in psychotherapy—in both kinds of relationships it is the interaction which provides the continuity of the self, even though the integrity of the self and its capacity for self-regulation and healing is given from the beginning. This new model that developed in the developmental school is fundamentally different from Jung's model, which puts emphasis on the aspect of self-regulation, in the sense that the process comes out of the individual; the focus is more on interactions with inner figures (e.g., anima). In Jung's model, the other is nothing more than a projection screen on which the archetypal deintegrates are projected, and, if the process is wholesome, can be acknowledged as parts of the self, which then can be taken back and incorporated consciously into

the personality. There is also another important point to be made: the model of the developmental school is more or less identical with the idea that from the beginning there is a preformatted identity or centre of the personality and that the process is of a centring nature. As far as I can see, there is no place in the developmental school for what I have called the classical archetypes, that is a process which can be mapped as a sequence of stages (shadow, anima/animus, the mana-personalities etc.) (Roesler, 2023).

From my point of view it is still an open question how these viewpoints on the therapeutic relationship (Jungian, post-Jungian and post-Freudian) can be integrated. So, for example, what do the findings of attachment research mean for an archetypal perspective on the therapeutic process? In Germany, as far as I can see, in the training institutes, a modern approach to the transference is presented, which is based on object relations theory, self-psychology and relational perspectives, and in a separated section there is teaching on archetypal transference, but it is not clear what the two have to do with each other. There is seemingly not even an awareness that the two models are, in a certain sense, contradictory.

Jean Knox (2009), in her paper which attempts to integrate Jungian, attachment theory and developmental perspectives into a theoretical model of the analytical relationship, correctly stresses the point that it was Jung, who first in the history of psychoanalysis spoke of the mutual relationship between analyst and patient, in which both descend into mutual unconscious entanglements and projections, out of which a conscious understanding and eventually individuation will emerge.

Nevertheless, there remain sharp divisions between different groups in both psychoanalysis and analytical psychology about the relative importance of the relational and interpretive aspects of analytic work. These divisions partly reflect the differing perceptions of the nature of the unconscious. (Knox, 2009, p. 6)

Although contemporary approaches in the psychodynamic psychotherapies, informed by attachment theory, strongly support the idea that was already present in Jung's thought, of a self-organizing principle in the psyche, these contemporary approaches differ from the classical viewpoint in Jungian psychotherapy when it comes to the role and shape of the therapeutic relationship.

It supports the view that the analytical relationship needs to be more flexible than either the classical psychoanalytic interpretive or the classical Jungian archetypal models would allow; in place of the uncovering of specific mental content (e.g., repressed Oedipal material or archetypes), an attachment orientated analyst accompanies the patient on the developmental journey, one that will sometimes require interpretation of such material but will also allow for new experiences to emerge in the analytical relationship. (Knox, 2009, pp. 8–9)

Knox points out that beginning with the object relations school in psychoanalysis, all of the contemporary relational approaches in psychoanalysis agree on the three fundamental developmental tasks that have to be accomplished in a successful therapy:

1. affect regulation
2. the capacity for mentalization
3. a sense of self agency

These capacities, especially affect regulation and the capacity for mentalization directly result from the relational interaction with the therapist/analyst: especially “the emotional regulation offered by the relationship creates the conditions necessary for the neuronal development in the orbitofrontal cortex and other areas on which affect regulation depends” (Knox, 2009, p. 10). Psychotherapeutic change comes about through the experience of being mentalized in an emotionally safe and valued relationship. In one of his latest papers, Peter Fonagy (Fonagy et al., 2022) even presents the viewpoint that all the psychological problems that need psychotherapy are rooted in experiences of being insufficiently mentalized in the course of development, and can, therefore, be treated effectively by a mentalization focused approach. Incidentally, this corresponds to what Shedler (2022) defines as the basic elements of modern psychodynamic psychotherapy: Focus on emotion, emotional expression; exploration of patient’s attempts to avoid distressing thoughts and feelings (defences); identification of recurrent patterns; exploration of past experiences; focus on interpersonal relationships; focus on therapy relationship; exploration of fantasies.

These tasks are very different from what Jung thought to be the aims of therapy. Here, the relational model speaks of mental capacities of emotion regulation, whereas Jung speaks of clearly defined stages of a process—which has a parallel in Jung’s view of biologically inherited qualities, i.e., archetypes, as being specified by their content (e.g., anima, the wise old man etc.) versus contemporary insights in innate qualities being mainly capacities for relating and interpersonal emotion regulation. Contemporary developmental models put the emphasis on capacities and not on content, whereas Jung’s idea was that content patterns were preformatted. It also has to be noted that, based on the research which found that inborn capacities consist of interaction and relationship abilities, in contemporary psychodynamic approaches the self in development is always conceptualized as “Self-being-with-other” (Stern, 1985)—which is fundamentally different from Jungian conceptualizations. Contemporary approaches see the relationship at the beginning, whereas the Jungian school assumes that the self is preformatted and primary. So, where does development come from, from the experience of the relationship with the therapist (which is then internalized as “good object”), or is the therapist and the therapeutic relationship just working as a catalyst for an autonomous

“preformatted” process coming from within? If we will not be able to point out what is specific about the Jungian approach in terms of how development comes about and how therapeutic change is effected, there is no reason why we should not just merge with the other psychodynamic schools—as publications like that by Knox (2009) imply.

Self-Righting

I would like to conclude with reference to a concept developed in the context of Kohut's self psychology, the capacity of self-righting (Milch, 2022). By this is meant an intrinsic tendency of the organism to recover from a deficit and to make a developmental advance even when external conditions were obstructive. A normal developmental step that was hindered or missing altogether becomes possible later. It is an intrinsic tendency that makes psychotherapy possible, i.e., moving from a pathologically altered, lower functioning state to a better adapted state, the ability to rebuild the self. For this to be possible, both the organism and the environment must contribute. It also includes the creative possibility of producing conditions in a relationship that can reverse a deficient development, for example, using the therapeutic relationship, which is consistent with the concept of corrective emotional relational experience. Obviously, there is a strong parallel to Jung's concept of the Self as an inherent capacity to reorganize the psyche in the form of a centring process:

As a numinous factor, the archetype determines the nature of the configurational process and the course it will follow, with seeming fore-knowledge, or as though it were already in possession of the goal to be circumscribed by the centring process. (Jung, 1969, para. 411)

But, again, here the focus is on making use of favourable conditions in the form of a specific relationship, in contrast to Jung's focus on individual autonomy. This difference also becomes apparent in the technical recommendations for psychotherapy to support self-righting (Milch, 2022). What is needed is:

- A framework of kindness, consistency, reliability, and a safe ambience;
- Systematic application of the empathic mode of perception/mentalization;
- Recognition of affect in order to validate the patient's experience;
- Interventions that communicate one's understanding, express affective attunement, and take the patient's point of view;
- Following the interaction with observation of the patient's affective states, especially when there is a rupture/empathic failure on the part of the analyst, the analyst takes responsibility for the interruption, independent of his/her original intention.

Again, here is the focus on a certain quality of the therapeutic relationship, instead of being on contents in the sense of symbols and images which appear in the process.

It was the aim of this paper to point out unresolved questions in analytical psychology which urgently need clarification. In general, there is a strong need to adapt the models of personality and therapy process in Jungian psychology to the developments in emotion theories and the neurosciences. To be more specific, a clarification is needed: what is the factor behind therapeutic change? An autonomous individuation process or the internalization of the therapeutic relationship in the sense of a corrective emotional relational experience? An answer to these questions is crucial for the future of analytical psychology.

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TRANSLATIONS OF ABSTRACT

Toutes les psychothérapies actuelles s'accordent sur le fait que la régulation (défaillante) de l'émotion est au centre des désordres psychologiques et que la psychothérapie vise à améliorer la régulation de l'émotion. Dans sa recherche sur le « complexe à haute charge émotionnelle », Jung a mis l'accent sur le rôle de la régulation défaillante de l'émotion comme participant aux désordres psychologiques ainsi qu'au changement dans le processus de psychothérapie. Mais il a abandonné ce champ de recherche et pris une direction très différente, y préférant son concept de l'archétype. Les approches psychodynamiques plaident généralement en faveur de l'idée que les changements dans la régulation de l'émotion sont atteints par les expériences émotionnelles corrective dans la relation thérapeutique. Des apports venant des neurosciences affectives et des recherches sur l'attachement ont eu une influence majeure sur comment la relation thérapeutique est construite dans les approches psychodynamiques actuelles. De tels développements font défaut dans la psychologie analytique, ce qui conduit à des différences considérables entre les modèles de psychothérapie jungienne en contraste avec les autres approches psychodynamiques actuelles. L'article souligne les conséquences de ces différences dans la pratique de la psychothérapie, tout particulièrement en ce qui concerne le rôle de la relation thérapeutique.

Mots clés: régulation de l'émotion, neurosciences affectives, recherche sur l'attachement, relation thérapeutique, psychothérapie psychodynamique, psychologie analytique

Alle zeitgenössischen Psychotherapien stimmen darin überein, daß (fehlende) Emotionsregulation für psychische Störungen von zentraler Bedeutung ist und daß es bei der Psychotherapie um die Verbesserung der Emotionsregulation geht. In seiner Forschung zum "emotionsbeladenen" Komplex legte Jung den Schwerpunkt auf die Rolle von fehlgeschlagener Emotionsregulation bei der Entstehung psychischer Störungen sowie bei Veränderungen im Prozeß der Psychotherapie, verließ aber dieses Forschungsgebiet und schlug eine ganz andere Richtung zugunsten seines Archetypenkonzeptes ein. Psychodynamische Ansätze argumentieren im Allgemeinen, daß Veränderungen in der Emotionsregulation durch korrigierende emotionale Erfahrungen in der therapeutischen Beziehung erreicht werden. Erkenntnisse aus der affektiven Neurowissenschaft und der Bindungsforschung haben großen Einfluß darauf, wie die therapeutische Beziehung in zeitgenössischen psychodynamischen Ansätzen konstruiert wird. Es mangelt an ähnlichen Entwicklungen in der Analytischen Psychologie, was zu erheblichen Unterschieden zwischen den Modellen der Jungianischen Psychotherapie und anderen zeitgenössischen psychodynamischen Ansätzen führt. Die Implikationen dieser Unterschiede für die Praxis der Psychotherapie und insbesondere die Rolle der therapeutischen Beziehung werden aufgezeigt.

Schlüsselwörter: Emotionsregulation, affektive Neurowissenschaften, Bindungsforschung, therapeutische Beziehung, psychodynamische Psychotherapie, Analytische Psychologie

Tutte le psicoterapie contemporanee concordano sul fatto che la (fallita) regolazione delle emozioni è centrale per i disturbi psicologici e che la psicoterapia riguarda il miglioramento della regolazione delle emozioni. Nella sua ricerca sul complesso “carico di emozioni”, Jung ha posto l’accento sul ruolo della fallita regolazione delle emozioni nel contribuire ai disturbi psicologici e al cambiamento nel processo della psicoterapia, ma ha poi abbandonato questo campo di ricerca ed ha preso una direzione molto diversa, centrata sul suo concetto di archetipo. Gli approcci psicodinamici generalmente sostengono che i cambiamenti nella regolazione delle emozioni avvengono tramite esperienze emozionali correttive nella relazione terapeutica. Le intuizioni delle neuroscienze affettive e della ricerca sull’attaccamento hanno avuto un’influenza importante sul modo in cui la relazione terapeutica è costruita negli approcci psicodinamici contemporanei. C’è una mancanza di analoghi sviluppi nella psicologia analitica, che porta a differenze sostanziali tra i modelli della psicoterapia junghiana, in contrasto con altri approcci psicodinamici contemporanei. Vengono qui sottolineate le implicazioni di queste differenze per la pratica della psicoterapia, ed in particolare il ruolo della relazione terapeutica.

Parole chiave: regolazione delle emozioni, neuroscienze affettive, ricerca sull’attaccamento, relazione terapeutica, psicoterapia psicodinamica, psicologia analitica

Все современные психотерапевты сходятся во мнении, что (недостаточная) регуляция эмоций является центральным фактором психологических расстройств и что психотерапия направлена на улучшение регуляции эмоций. Исследуя “эмоционально нагруженный” комплекс, Юнг подчеркивал роль нарушения эмоциональной регуляции в развитии психологических расстройств, а также изменения в процессе психотерапии, однако он отказался от изучения этой сферы в пользу совершенно иного направления исследования – концепции архетипа. Для психодинамических подходов характерно представление, что изменения в регуляции эмоций достигаются посредством корректирующих эмоциональных переживаний в терапевтических отношениях. Данные, полученные в нейробиологии эмоций и исследованиях привязанности, оказали значительное влияние на характер построения терапевтических отношений в современных психодинамических подходах. Аналитической психологии недостает аналогичных исследований, вследствие чего между моделями юнгианской психотерапии, в отличие от других современных психодинамических подходов, имеются существенные различия. Показано, как отражаются эти различия на практике психотерапии и, в частности, на представлениях о роли терапевтических отношений.

Ключевые слова: регуляция эмоций, нейробиология эмоций, исследование привязанности, терапевтические отношения, психодинамическая психотерапия, аналитическая психология

Todas las psicoterapias contemporáneas coinciden en que la regulación (fallida) de las emociones es central a los trastornos psicológicos y que la psicoterapia consiste en

mejorar la regulación de las emociones. En su investigación sobre el complejo "de tonalidad afectiva", Jung hizo hincapié en el rol de una fallida regulación emocional en el desarrollo de los trastornos psicológicos, así como al cambio en el proceso de psicoterapia, pero abandonó este campo de investigación y tomó una dirección muy diferente en favor de su concepto de arquetipo. En general, los enfoques Psicodinámicos sostienen que los cambios en la regulación de las emociones se logran a través de experiencias emocionales correctivas en la relación terapéutica. Los conocimientos de las neurociencias afectivas y la investigación sobre el apego han tenido una gran influencia en cómo comprender la conformación de la relación terapéutica en los abordajes psicodinámicos contemporáneos. Faltan desarrollos similares en la psicología analítica, lo que conduce a diferencias sustanciales entre los modelos de la psicoterapia Junguiana en contraste con otros enfoques psicodinámicos contemporáneos. Se señalan las implicaciones de estas diferencias para la práctica de la psicoterapia y se destaca especialmente el rol de la relación terapéutica.

Palabras clave: regulación emocional, neurociencia afectiva, investigación sobre el apego, relación terapéutica, psicoterapia psicodinámica, psicología analítica

心理治疗中情绪的角色

所有当代心理疗法都认为, (失效的)情绪调节是心理障碍的核心, 而心理疗法就是要改善情绪调节。荣格在研究 "情绪负荷" 情结时, 强调了情绪调节失灵在导致心理障碍以及在心理治疗过程中对促进改变所起的作用, 但他离开了这一研究领域, 转而采用了与此截然不同的研究方向, 原型概念。心理动力学方法一般认为, 治疗关系中矫正性的情绪体验, 可以带来情绪调节的改变。情绪神经科学和依恋研究对当代心理动力学方法如何构建治疗关系产生了重大影响。在分析心理学中却缺乏类似的发展, 这导致荣格心理治疗模式与其他当代心理动力学方法之间存在着巨大差异。本文指出了这些差异对心理治疗实践的影响, 尤其是对治疗关系的作用的影响。

关键词: 情绪调节, 情绪神经科学, 依恋研究, 治疗关系, 心理动力学心理疗法, 分析心理学
